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ascp
American Society of Consultant Pharmacists

June 16, 2025

The Honorable Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services
Department of Health & Human Services
200 Independence Ave SW,
Washington, DC 20201

RE: FR Doc. 2025-08701 Request for Information; Health Technology Ecosystem – **File Code: CMS-0042-NC**

Dear Administrator Oz:

On behalf of the American Society of Consultant Pharmacists (ASCP), we are pleased to submit the following comments in response to the Center for Medicare and Medicaid Services (CMS) Request for Information (RFI) regarding the digital health product market for Medicare beneficiaries, as well as data interoperability and health technology infrastructure.

ASCP is the only international professional society devoted to optimal medication management and improved health outcomes for older adults. Our thousands of pharmacist members manage drug therapies and support the quality of life of geriatric patients and others living in a wide variety of care settings, including: sub-acute and long-term care facilities (LTCFs), skilled nursing facilities (SNFs), assisted living facilities, psychiatric hospitals, hospice programs, correctional facilities, and home and community-based care.

Given the number of different facility types served by our members and their practice focus on senior care, ASCP member pharmacists are well positioned to support HHS with our expertise in medication management, transitions of care and chronic disease management. Our members are glad to provide feedback on barriers to digital health adoption, interoperability and provider access to patient health data.

In response to the RFI, ASCP is providing input related to the following topics outlined in two broad sections: "Patients and Caregivers" and "Providers."

Patients and Caregivers

It is a simple reality that patients have better health outcomes when their needs and preferences are respected and central to care. Recently, ASCP has proudly focus a significant amount of our training on the 4Ms of age-friendly care. The "Ms" of the program represent: "what matters," "medication," "mentation" and "mobility;" while placing "what matters" as the central element in the care framework.[i] We are pleased to see HHS and CMS placing "what matters" for individuals covered by Medicare front and center in this RFI.

PC-1: Pharmacy practice is an integral part of the health care system; ASCP supports the use of FDA-approved, interoperable health information technology (health IT), which includes digital health apps and electronic health records (EHR), to improve patient safety and outcomes, provide better and more timely access to health information, and integrate pharmacy health IT into the national health IT infrastructure. It is critical that health IT, as well as future CMS and ASTP/ONC proposals, enable pharmacists to improve public health, prevent chronic disease progression, and ensure that electronic health information is available to support the pharmacist's role in health care delivery. In this regard, ASCP encourages CMS and ASTP/ONC to examine the Pharmacist eCare PlanStandard Medication Profile (SMP).[ii]

ASCP participated in the development of the eCare Plan, "an interoperable standard that allows for pharmacy technology providers to have a common method of exchanging information related to care delivery, including patient goals, health concerns, active medication list, drug therapy problems, laboratory results, vitals, payer information, and billing for services."[iii] Care planning is an essential part of patient-centered care services provided by pharmacists. The eCare Plan allows pharmacists to identify and address a patient's health care barriers.[iv]

The HL7 FHIR SMP Implementation Guide "defines a way to transmit medication/drug information (as prescription or orders or records of a patient receiving some kind of medication/drug). As a patient travels through the healthcare system (such as from hospital to nursing facility to home) this guide shows how the medication/drug information can be sent electronically with the patient to enable medication reconciliation."[v] Ensuring an updated and accurate patient's medication profile is available to providers in transition of care and emergency situations is essential to patient safety.

PC-3/4: ASCP is aware of a variety of apps that seek to support older adults and their caregivers with managing, often sensitive, medical information. Given our practice experience, ASCP supports the integration of FDA-cleared digital health apps that support medication adherence, chronic disease monitoring, and deprescribing tools into the Medicare ecosystem.

While our pharmacists practicing in community and assisted living settings often encounter these apps, there is no standardization, interoperability and many leave out critical information, such as current medication lists. ASCP is supportive of voluntary efforts by industry and/or incentive models from CMS that explore the development and/or improvement of these apps. Should CMS pursue the development of a model app, we believe that extensive and targeted marketing to individuals covered by Medicare and their caregivers would be essential to wide adoption. Additionally, we remind developers that digital health tools for Medicare beneficiaries must be designed with accessibility and simplicity in mind, especially for older adults with limited digital literacy, vision issues, or cognitive impairment.

PC-14.b: As discussed above, ASCP pharmacists have extensive expertise in managing transitions of care, beyond the critical task of reconciling the medication list. While we are aware of the concerns related with a Universal Patient Identifier (UPI,) our recent experiences with managing COVID-19 immunization records outlines the benefits of a universal system that allow providers to access a patient's full medical records, from any location, at any time. By way of a

brief example, please consider the case of "snowbirds" moving around the U.S. throughout the year. At present, immunization recording systems (IIS) are managed by each state. As "snowbirds" moved between states, it became difficult to reconcile the patient's immunization record because vaccinations received in Florida were only recorded in Florida's IIS and therefore inaccessible to providers in Connecticut. Therefore, ASCP encourages CMS to consider mechanisms to improve communication and interoperability amongst IIS, as well as other state-managed medical record systems.

Providers:
PR-2: Although health IT has advanced, barriers exist and affect adoption, access by patients and providers, security, and privacy. The government has undertaken EHR adoption efforts, including providing incentives; yet interoperability remains unachieved. EHR systems must be able to share information between all relevant systems.

Pharmacies and the long-term care sector have been slow to fully adopt EHR, in part, because of the cost involved. For example, neither pharmacies nor LTC facilities were not included within the *HITECH Act*'s incentive program.

Ironically, as one of the most accessible health care providers in the country, pharmacists were deemed ineligible for the CMS EHR Incentive Program (now called Medicare Promoting Interoperability Program) because they are not defined as health care providers under Section 1171 of the *Social Security Act* – the governing law used for CMS programs. This oversight needs to be corrected, and pharmacists need to be recognized as providers by the federal government as they are by many Medicaid programs and private/commercial health plans across the country. This is a significant, but easily removed, barrier that must be addressed.

Specific to the long-term care sector, ASCP reminds HHS and CMS that previous efforts included significant financial incentives for many providers, but SNFs and other LTC settings were not eligible for these incentives. We urge CMS to take a comprehensive approach to the transfer of health information by focusing on additional efforts to advance interoperability across the care continuum, including SNFs, via electronic data exchange.

Ensuring interoperability across electronic health record (EHR) systems and settings of care can unlock barriers to data sharing and care coordination between health systems, physician group practices, independent physicians, and post-acute care (PAC) settings. Additional barriers for PAC providers to adopt EHR systems include a lack of financial incentives under the *Health Information Technology for Economic and Clinical Health (HITECH) Act* and no mandated EHR adoption requirements. As a result, many SNFs and other PAC providers are not using EHRs or are using EHRs that are not designed for interoperability.

ASCP urges CMS to enhance its efforts to develop standards and measures for data exchange and sharing across all care settings, including SNFs. The transfer of information between providers most often occurs via cumbersome and resource-intensive manual processes. CMS should consider ways to incentivize PAC providers to more readily adopt health IT in support of

wider efforts to standardize patient data, improve care quality, and reduce costs. Standardized data elements and common data reporting processes alone will not achieve interoperability across the care continuum.

PR-5: As a member of the Pharmacy Health Information Technology (PHIT) Collaborative, ASCP is an active participant with Health Level Seven International (HL7) and supports all the FHIR APIs (Fast Healthcare Interoperability Resources Application Programming Interfaces) listed in PR-5 (a-i).

PR-13: On November 1, 2023, CMS and ASTP/ONC published "21st Century Cures Act: Establishment of Disincentives for Health Care Providers That Have Committed Information Blocking," a proposed rule that also requested information for future rulemaking on additional appropriate disincentives that should be considered and applied to health care providers not implicated in the proposed rule.

Through comments submitted as part of PHIT on January 2, 2024 comments, we recommended that CMS and ASTP/ONC not proceed with establishing or applying disincentives for pharmacies until such EHR requirements and criteria for interoperability are in place for pharmacies.

Although a pharmacist and a pharmacy are defined in the health care provider term (42 U.S.C. 300-jj) used in the CMS/ASTP/ONC proposed rule, the disincentives for information blocking outlined in the proposal cannot be applied to them, as they are not participants in the EHR Incentive Program for which the proposed rule applies. Pharmacists and pharmacies are not defined as meaningful users of CEHRT (certified electronic health record technology) in the Merit-Based Incentive Payment System (MIPS) program, as they are not defined as eligible professionals nor as meaningful EHR users in the EHR Incentive Program by CMS. Since the creation of this program in 2011, ASCP and other pharmacy associations have requested the Secretary as well as Congress to correct this omission to include pharmacists as health care providers under Section 1171 of the *Social Security Act*.

A pharmacist's or pharmacy's certification and use of certified EHR in the Merit-based Incentive Payment System (MIPS) Promoting Interoperability Program is currently voluntary because they do not receive incentives from the EHR Incentive Program for demonstrating use of CEHRT. Until EHR requirements and criteria for interoperability are identified for a pharmacist or a pharmacy, disincentives cannot be applied; only the reporting of possible information blocking by a pharmacist or pharmacy is allowed.

Finally, ASCP encourages HHS to expand its education efforts on informing the public about what rights they have with their own health data, what information blocking is, and why it is so critical for patients and caregivers to report entities that are interfering with the access of pertinent health information.

Conclusion

ASCP urges HHS and CMS to prioritize long-term care settings, the consultant pharmacists who work there, and the geriatric patients who receive care there in the expansion of digital health infrastructure. Consultant pharmacists are already embedded in interdisciplinary teams, and with better access to interoperable health data, they can improve medication safety, reduce overall healthcare costs, and enhance health outcomes for Medicare beneficiaries.

We stand ready to work with the Department to ensure all older Americans enjoy the benefits of a modern, integrated digital health ecosystem that truly meets their needs. For further questions or follow-up, please contact James Lewis at jlewis@ascp.com.

Sincerely,

James Lewis
Senior Director of Policy & Advocacy
American Society of Consultant Pharmacists (ASCP)

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[i] https://www.johnahartford.org/resources/view/american-society-of-consultant-pharmacists-course-leveraging-pharmacists-as-age-friendly-4ms-champions-home-study
[ii] https://www.ncpdp.org/NCPDP/media/pdf/Pharmacist-eCare-Plan.pdf
[iii] https://www.ecareplaninitiative.com/
[iv] https://www.pharmacyowners.com/the-pharmacist-ecare-plan
[v] https://www.hl7.org/fhir/us/smp/index.html